Supplementary Table S1 - Overview of reported *C. striatum* cases.

Article	Study design	Attrition/Selective reporting bias	Age (years)	Sex	Site of isolation of *C. Striatum*	Type of infection	Antibiotic used for therapy	Dose of antibiotic used for therapy	Duration of antibiotic therapy	Outcomes of antibiotic treatment
Abi *et al.*[12]	Case report	N/A	51	M	Blood	Endocarditis	Ciprofloxacin + vancomycin	Not reported	6 weeks	Clinical Improvement
Beltrán-Arroyave *et al.*[13]	Case report	N/A	13	F	Tissue and bone samples	Ostemyelitis	Vancomycin + Linezolid	Vancomycin IV+ linezolid orally	Vancomycin 35 days; Linezolid 14 days	Complete recovery
Bhandari *et al.*[14]	Case report	N/A	53	M	Continuous Ambulatory Peritoneal Dialysis fluid	Continuous Ambulatory Peritoneal Dialysis peritonitis	Vancomycin + Ceftazidime	Vancomycin (50 mg per exchange) ceftazidime (1 g of loading dose, then 250 mg per exchange) IP	10 days	Complete recovery
Biswal *et al.*[15]	Case report	N/A	57	M	Pus	Laryngeal fistula infection	Vancomycin	1 g/12 h	Unknown	Clinical Improvement
Chatzopoulou *et al.*[16]	Case report	N/A	76	M	Blood	Sepsis	Ceftriaxone + Moxifloxacin + Linezolide; switched to Vancomycin	Not reported	Ceftriaxone + Moxifloxacin + Linezolid - 3 days; Vancomycin- 1 day	Death
Cone *et al.*[17]	Case report	N/A	51	M	Swab wounds	Septic arthritis	Vancomycin, switched to Ciprofloxacin	Not reported	Vancomycin 10 days; Ciprofloxacin 7 days	Complete recovery
Cowling *et al.*[18]	Case report	N/A	69	M	Sputum	Exacerbation of chronic obstructive airway disease	Amoxicillin	Not reported	Unknown	Clinical Improvement
Daisuke *et al.*[19]	Case report	N/A	49	F	Blood and central venous catheter	Bacteremia Associated with a Catheter-Related Blood Stream Infection	Vancomycin + Meropenem	Vancomycin 1 g/6 h Meropenem 1 g/6 h	5 days	Death
Elshibly *et al.*[20]	Case report	N/A	77	F	Blood	Endocarditis	Unknown	Not reported	58 days	Complete recovery
Fernández Guerrero *et al.*[21]	Case report	N/A	78	M	Blood	Endocarditis+ spondylodiscitis+eepidural abscess	Daptomycin	10 mg/kg (750 mg every other day)	7 weeks	Clinical Improvement
Fernández-Esgueva *et al.*[22]	Case report	N/A	85	F	Joint fluids and sample from prosthesis	Prosthetic joint infection	Vancomycin+ceftazidime, swithed to linezolid	Vancomycin 1 g/12 h IV + Ceftazidime 2 g/8 h, swithed to Linezolid 600 mg/12 h	Vancomycin + Ceftazidime - 5 days; Linezolid 9 days	Clinical Improvement
Gehlhausen *et al.*[23]	Case report	N/A	68	M	Blood	Botryomycosis-like tissue infection	Vancomycin + Piperacillin/ tazobactam	Not reported	Unknown	Death
Houghton *et al.*[24]	Case report	N/A	62	F	Blood	Endocarditis	Vancomycin	Not reported	7 weeks	Complete recovery
Iaria *et al.*[45]	Case series	Attrition bias- low risk	73	M	Bronchial aspirate	Unknown	Teicoplanin	Not reported	Unknown	Death
Iaria *et al.*[45]	Case series	Attrition bias- low risk	16	M	Central venous catheter	Unknown	Ceftazidime	Not reported	Unknown	Complete recovery
Iaria *et al.*[45]	Case series	Attrition bias- low risk	59	M	Bronchial aspirate	Unknown	Teicoplanin	Not reported	Unknown	Complete recovery
Iaria *et al.*[45]	Case series	Attrition bias- low risk	16	F	Blood	Unknown	Teicoplanin + Meropenem	Not reported	Unknown	Death
Iaria *et al.*[45]	Case series	Attrition bias- low risk	33	M	Bronchial aspirate	Unknown	Teicoplanin	Not reported	Unknown	Complete recovery

Supplementary Table S1 - Overview of reported *C. striatum* cases. (cont.)

Article	Study design	Attrition/Selective reporting bias	Age (years)	Sex	Site of isolation of *C. Striatum*	Type of infection	Antibiotic used for therapy	Dose of antibiotic used for therapy	Duration of antibiotic therapy	Outcomes of antibiotic treatment
Iaria *et al.*[45]	Case series	Attrition bias- low risk	80	F	Blood	Unknown	Piperacillin + Teicoplanin	Not reported	Unknown	Complete recovery
Iaria *et al.*[45]	Case series	Attrition bias- low risk	69	M	Bronchial aspirate	Unknown	Linezolid + Levofloxacin	Not reported	Unknown	Complete recovery
Iaria *et al.*[45]	Case series	Attrition bias- low risk	55	F	Bronchial aspirate	Unknown	Teicoplanin	Not reported	Unknown	Death
Jagadeeshan *et al.*[25]	Case report	N/A	27	M	Blood	Endocarditis	Cefuroxime	2 g IV	Unknown	Clinical Improvement
Markowitz *et al.*[26]	Case report	N/A	76	M	Blood	Endocarditis	Ampicillin + Gentamicin	Not reported	Unknown	Death
Marull *et al.*[27]	Case report	N/A	73	F	Blood	Endocarditis	Vancomycin	Not reported	6 weeks	Clinical Improvement
Roy *et al.*[28]	Case report	N/A	77	M	Synovial fluid aspirate	Septic arthritis	Vancomycin	Not reported	4 months	Clinical Improvement
Rufael *et al.*[29]	Case report	N/A	54	M	Blood	Endocarditis	Vancomycin + Ampicillin, switched to Ampicillin + Gentamicin, switched to Vancomycin	Not reported	Ampicillin + Vancomycin 2.5 weeks, Total antibiotic treatment - 6 weeks	Clinical improvement
Saito *et al.*[30]	Case report	N/A	47	F	Blood	Bacteremia Associated with a Catheter-Related Blood Stream Infection	Vancomycin, switched to oral Amoxicillin	Vancomycin 1g/12 h, switched to oral amoxicillin (not reported)	Total antibiotic duration 22 days; Amoxicillin 14 days	Complete recovery
Savini *et al.*[31]	Case report	N/A	60	ND	Blood	Bacteremia	Vancomycin	Not reported	48 hours	Clinical Improvement
Scholle[32]	Case report	N/A	87	M	Synovial fluid	Spontaneous joint infection	Vancomycin + Bacitracin	Not reported	2 weeks (bacitracin was used only for flushing joint place)	Clinical improvement
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	72	F	Ulcer aspirate	Leg ulcer infection, sepsis	Cefazolin	2 g IV/8h	Not specified	Death
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	59	M	Wound aspirate	Surgical wound infection	Vancomycin	1 g/12 h	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	64	F	Bronchial aspirate	Bronchopneumonia	Vancomycin	1 g/12 h	2 days	Death
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	66	M	Wound aspirate	Surgical wound infection	Ampicillin	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	52	M	Bronchial aspirate	Bronchopneumonia	Teicoplanin	Not reported	Not specified	Death
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	27	M	Pleural fluid	Pleural empyema	Metronidazole	Not reported	Not specified	Death
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	78	F	Ulcer aspirate	Leg ulcer infection	Ampicillin	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	30	M	Ulcer aspirate	Decubitus ulcer infection	Vancomycin	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	54	M	Bronchial aspirate	Bronchitis	Vancomycin	Not reported	Not specified	Complete recovery

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Article	Study design	Attrition/Selective reporting bias	Age (years)	Sex	Site of isolation of *C. Striatum*	Type of infection	Antibiotic used for therapy	Dose of antibiotic used for therapy	Duration of antibiotic therapy	Outcomes of antibiotic treatment
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	69	M	IV catheter	IV catheter related infection	Aztreonam + Vancomycin	Not reported	Not specified	Death
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	22	M	Wound aspirate	Surgical wound infection	Aztreonam	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	59	M	Bronchial aspirate	Bronchitis	Vancomycin	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	67	M	Wound aspirate	Surgical wound infection	Vancomycin	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	84	M	Ulcer aspirate	Leg ulcer infection	Ampicillin	Not reported	Not specified	Complete recovery
Martínez-Martínez *et al.*[5]	Case series	Attrition bias- medium risk	75	M	Ulcer aspirate	Leg ulcer infection	Vancomycin	Not reported	Not specified	Clinical Improvement
Noussair *et al.*[9]	Case series	Attrition bias- low risk	67	F	Intraoperative samples of bone and joint infection	Osteomyelitis	Amoxicillin-clavulanate + Ciprofloxacin, switched to Amoxicillin + Rifampicin	Not reported	50 days	Failure of antibiotic therapy (amputation)
Noussair *et al.*[9]	Case series	Attrition bias- low risk	65	F	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Amoxicillin + Rifampicin	Not reported	6 weeks	Failure of antibiotic therapy (reccurrence)
Noussair *et al.*[9]	Case series	Attrition bias- low risk	70	F	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Vancomycin + Gentamycin, switched to Daptomycin + Rifampicin	Not reported	7 weeks (total)	Complete recovery
Noussair *et al.*[9]	Case series	Attrition bias- low risk	49	M	Intraoperative samples of bone and joint infection	Osteomyelitis	Piperacillin-Tazobactam + Vancomycin, switched to Amoxicillin + Rifampicin	Not reported	5 weeks	Complete recovery
Noussair *et al.*[9]	Case series	Attrition bias- low risk	67	F	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Piperacillin-Tazobactam + Daptomycin, switched to Amoxicillin + Rifampicin	Not reported	5 weeks	Complete recovery
Noussair *et al.*[9]	Case series	Attrition bias- low risk	70	F	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Piperacillin-Tazobactam + Daptomycin, switched to Amoxicillin + Rifampicin	Not reported	6 weeks	Complete recovery
Noussair *et al.*[9]	Case series	Attrition bias- low risk	24	M	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Piperacillin-Tazobactam + Vancomycin, switched to Amoxicillin + Rifampicin	Not reported	6 weeks	Complete recovery
Noussair *et al.*[9]	Case series	Attrition bias- low risk	77	F	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Piperacillin-Tazobactam + Daptomycin, switched to Amoxicillin + Amoxicillin-clavulanate + Ofloxacin	Not reported	5 weeks	Complete recovery
Noussair *et al.*[9]	Case series	Attrition bias- low risk	87	M	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Piperacillin-Tazobactam + Daptomycin, switched to Amoxicillin + Rifampicin	Not reported	5 weeks	Death
Noussair *et al.*[9]	Case series	Attrition bias- low risk	82	M	Intraoperative samples of bone and joint infection	Infection of prosthetic material	Piperacillin-Tazobactam + Daptomycin, switched to Amoxicillin + Rifampicin	Not reported	2.5 months	Failure of antibiotic therapy (superinfection)
Noussair *et al.*[9]	Case series	Attrition bias- low risk	47	F	Intraoperative samples of bone and joint infection	Bone and joint infection	Piperacillin-Tazobactam + Vancomycin switched to Daptomycin	Not reported	1 month	Complete recovery

Supplementary Table S1 - Overview of reported *C. striatum* cases. (cont.)

Article	Study design	Attrition/Selective reporting bias	Age (years)	Sex	Site of isolation of *C. Striatum*	Type of infection	Antibiotic used for therapy	Dose of antibiotic used for therapy	Duration of antibiotic therapy	Outcomes of antibiotic treatment
Severo *et al.*[1]	Case report	N/A	72	F	Lung biopsy sample	Multiple pulmonary nodules	Erythromycin + Rifampicin	Erythromycin (500 mg/ qds) Rifampicin (600 mg/ daily)	10 days	Death
Shariff *et al.*[47]	Case series	Attrition bias- low risk	59	M	Sputum	Exacerbation of COPD	Linezolid	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	66	M	Sputum	Exacerbation of COPD,	Piperacillin-Tazobactam + Levofloxacin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	73	M	Sputum	Exacerbation of COPD,	Amoxicillin-Clavulanic Acid + Azithromycin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	67	M	Sputum, Nasal swab	Exacerbation of COPD	Piperacillin-Tazobactam + Azithromycin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	55	M	Sputum, Nasal swab, Throat swab	Exacerbation of COPD	Piperacillin-Tazobactam + Levofloxacin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	74	M	Sputum	Exacerbation of COPD	Piperacillin-Tazobactam + Azithromycin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	68	M	Sputum	Exacerbation of COPD (Emphysema)	Piperacillin-Tazobactam + Azithromycin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	71	M	Tracheal aspirate	Exacerbation of COPD	Unknown	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	55	M	Tracheal aspirate	Unilateral pneumonia,	Piperacillin-Tazobactam + Azithromycin + Clindamycin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	72	M	Sputum	Bilateral pneumonia,	Piperacillin-Tazobactam + Levofloxacin	Not reported	Unknown	Complete recovery
Shariff *et al.*[47]	Case series	Attrition bias- low risk	70	M	Sputum	Exacerbation of COPD	Piperacillin-Tazobactam + Azithromycin	Not reported	Unknown	Complete recovery
Stone *et al.*[33]	Case report	N/A	41	F	Excised necrotic breast tissue	Recurrent breast abscess	Oxytetracycline	500 mg/qds IV	6 weeks	Complete recovery
Superti *et al.*[34]	Case report	N/A	27	M	Skin lesion biopsy	Progressive ulcerative skin lesion	Penicillin G crystalline	3 million units/4 h IV	15 days	Complete recovery
Tarr *et al.*[35]	Case report	N/A	58	M	Sputum and bronchoalveolar lavage fluid	Pneumonia	Vancomycin + Rifampicin	Not reported	7 days	Complete recovery
Tran *et al.*[36]	Case report	N/A	56	M	Blood	Sepsis	Telavancin	10 mg/kg every 48 h IV	3 weeks	Death
Tumbarello *et al.*[37]	Case report	N/A	26	M	Blood	Sepsis	Teicoplanin	400 mg/day IV	3 weeks	Complete recovery
Weiss *et al.*[38]	Case report	N/A	23	M	Cerebrospinal fluid	Meningitis	Ceftriaxone (disc after 1 day) and vancomycin	Ceftriaxone 1 g/12 h Vancomycin 1 g/12 h IV	10 days	Complete recovery
Westblade *et al.*[39]	Case report	N/A	84	M	Knee synovial fluid aspirate	Septic Arthritis	Vancomycin	1 g/12 h	4 weeks	Complete recovery
Yang *et al.*[2]	Case report	N/A	51	M	Blood and central venous catheter tip	Central Venous Catheter-Related Bloodstream Infection	Vancomycin	Not reported	40 days	Unknown
Yoo *et al.*[40]	Case report	N/A	64	M	Blood	Bloodstream Infection	Piperacillin-Tazobactam	Not reported	12 days	Unknown

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Article	Study design	Attrition/Selective reporting bias	Age (years)	Sex	Site of isolation of *C. Striatum*	Type of infection	Antibiotic used for therapy	Dose of antibiotic used for therapy	Duration of antibiotic therapy	Outcomes of antibiotic treatment
Selek *et al.*[41]	Case report	N/A	88	F	Blood, sputum, bronchoalveolar lavage	Pneumonia and bacteremia	Linezolid	2x600 mg IV	2 weeks	Complete recovery
Tibrewala *et al.*[42]	Case report	N/A	69	F	Blood	Native Valve Endocarditis	Vancomycin	Not reported	6 weeks	Complete recovery
Keijman *et al.*[43]	Case report	N/A	65	M	Blood	Native Valve Endocarditis, pneumonia,	Ticarciline + Gentamicin + Penicilin	Not reported	12 days	Complete recovery
Vandenbos *et al.*[44]	Case report	N/A	74	F	Sputum	Bronchial infection	Pristinamycin	3 x 1 g/day	10 days	Complete recovery
Werth *et al.*[46]	Case series	Attrition bias- low risk	67	M	Blood, drainage of an abscess	Left ventricular assist device infection	Daptomycin switched to Linezolid	Not reported	4 monts	Complete recovery
Watkins *et al.*[48]	Case series	Attrition bias- low risk	19	M	Peripheral blood and blood from a central venous catheter	Central-venous catheter-related bacteremia	Amikacin + Vancomycin	Not reported	4 weeks	Complete recovery
Watkins *et al.*[48]	Case series	Attrition bias- low risk	46	F	Central venous catheter tip	Central-venous catheter-related sepsis	Mezlocillin + Tobramycin + Vancomycin	Not reported	Unknown (Until recovery)	Complete recovery
Watkins *et al.*[48]	Case series	Attrition bias- low risk	31	F	Uterus, placenta, urine	Urinary tract infection, chorioamnionitis	Ampicillin-sulbactam	Not reported	1 week	Complete recovery
Verroken *et al.*[49]	Case series	Attrition bias- low risk	62	M	Chest drain, respiratory sample	Pneumonia, Pleural empyema	Cefuroxime-axetil + Piperacillin-Tazobactam	Not reported	Unknown (Until recovery)	Complete recovery

N/A = Not applicable; IV = Intravenous; ND = Not defined.